

Surgery for head and neck cancer

Surgery is one of the main treatments for head and neck cancer. Some people have treatment with chemoradiation or radiotherapy after surgery.

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About surgery for head and neck cancer

Surgery for [head and neck cancer](#) aims to remove the cancer completely. The type of operation you have depends on the size and position of the cancer and its [stage](#). You may have surgery:

- to completely remove the tumour and any tissue it has spread to
- to remove lymph nodes in the neck.

If the cancer or lymph nodes have been removed, tissue may be taken from another part of the body to replace tissue that was taken away. This is called reconstructive surgery.

Before your operation, your surgeon or nurse will explain what is involved and what to expect. For example, they will explain:

- how much tissue or bone they plan to remove
- how they plan to repair or replace any affected tissue or tissue that is removed
- the possible side effects from the operation.

Your surgeon will do everything possible to minimise changes to your [speech](#), swallowing, breathing and appearance. They will explain how the surgery may affect you in the short and long term

Before your operation

You will see different people from your [head and neck team](#) to help you [prepare for cancer treatment](#). They will give you advice on getting ready for surgery. It is important to follow this advice.

You usually have an appointment at a pre-operative assessment clinic. [Tests](#) to check your general health may include:

- blood and urine tests
- chest x-rays
- a recording of your heart (ECG).

You go into hospital on the day of your operation or the day before. The nurses give you elastic stockings (TED stockings) to wear during and after surgery, to help prevent blood clots.

Surgery to remove the tumour

If you have a very small cancer, it might be treated with a simple operation or laser surgery. The surgeon may operate through the open mouth. This means you will not have any external scars. You may be able to go home the same day as the surgery.

Some people also have surgery to remove lymph nodes in the neck (called neck dissection). Most operations to remove the tumour are done under a general anaesthetic.

Open surgery

If the cancer is bigger, or at the back of the mouth or in the throat, the surgeon usually needs to make a cut (incision) in the skin to reach it. This means you will have a scar afterwards. This usually fades over time.

Transoral surgery

This is a simple operation where the surgeon removes the tumour through the open mouth using different techniques. This type of surgery is usually used to treat small cancers in the mouth, tongue, throat or tonsils. It may help to reduce the risks of certain side effects, such as swallowing difficulties, and help you to recover faster. You will not have any external cuts or scars afterwards.

Transoral laser surgery (TLM)



The surgeon uses a high-powered beam of laser light to remove the cancer and a small area of healthy tissue around it. They examine the area closely using a microscope attached to the laser before removing the cancer.

Transoral robotic surgery (TORS)



The surgeon controls robotic arms attached to a fine instrument to remove the cancer through the mouth. It can allow the surgeon to do very exact surgery to remove the whole tumour.

Neck dissection (removing the lymph nodes in the neck)

Surgery to remove the lymph nodes in one or both sides of the neck is called a neck dissection. This surgery:

- tells your doctor more about the stage of the cancer
- removes any lymph nodes that contain cancer cells
- reduces the risk of the cancer coming back (if the lymph nodes contained cancer cells).

You have tests first to see if there are signs of cancer in the lymph nodes. If the cancer is large, your doctor may advise neck dissection surgery even if your test results are negative. With bigger cancers, there may be small amounts of cancer in the lymph nodes. This may not show up on scans or in other tests.

You may have a neck dissection:

- at the same time as surgery to remove the primary cancer
- before or after [chemoradiation](#) or [radiotherapy](#), if there are still cancer cells in the lymph nodes
- if the cancer is still present or comes back in the nodes after treatment.

After a neck dissection

After a neck dissection, possible side effects may include:

Weakness in your lower lip



The nerve responsible for moving the lower lip can sometimes be affected. This may be noticeable on one side of your mouth when you smile. It usually returns to normal after a few months.

Shoulder problems



The nerve that helps control shoulder movement may also be affected, causing shoulder stiffness. You usually see a physiotherapist who will give you exercises to help with this.

Neck problems



Your neck may feel stiff and tight after a neck dissection, especially if you have radiotherapy afterwards. You will be given exercises to reduce the risk of neck and shoulder problems.

Rarely, other nerves connected to the tongue and voice box can also be affected. Sometimes there may be a change in the shape of your neck.

Your surgeon and nurse will explain more about the possible side effects of a neck dissection. We have more information about [neck and shoulder problems](#) after treatment.

Reconstructive surgery

Your operation may involve removing tissue that is used for speech or swallowing. It may also affect your appearance. You may have reconstructive surgery at the same time as removing the tumour. The aim is to cause as little change as possible to the way everything looks and works.

The surgeon takes tissue from another part of the body, such as the forearm, thigh or chest. They use it to replace tissue taken from the head and neck. This is called a myocutaneous flap. Myo stands for muscle, and cutaneous stands for skin.

If the cancer is in the jawbone, they will need to remove the affected bone. The surgeon may use bone from another part of the body (usually the lower leg) to replace the missing jawbone. This is called a bone flap. Your doctor and nurse will give you more information about this operation. You can usually move your jaw again as soon as the surgery finishes.

Before an operation that is likely to change your appearance, it may help to talk to someone who has had a similar operation. They can tell you what helped them to cope. Your surgeon or specialist nurse may be able to arrange this for you.

Sometimes an area of bone, such as [the hard palate](#), needs to be removed. If that happens, the surgeon may advise that you have a false part (prosthetic) fitted to fill the space. This is not usually necessary. But if you need this, your surgeon and nurse will explain it to you carefully before surgery.

We have more information about [coping with changes to how you look](#).

Tracheostomy

Some people may need to have a small opening made in their windpipe during surgery. This is called a [tracheostomy](#). It helps with your breathing after surgery and is usually removed a few days later. Your surgeon and nurse will tell you if you may need a tracheostomy. They will explain what to expect.

After the operation

Although some people have surgery as a day patient, head and neck surgery often involves a stay in hospital. This may be for several days or up to a few weeks. This depends on the type of operation you have and whether you need reconstructive surgery.

After your operation, you will be encouraged to start moving around as soon as possible. This is an important part of your recovery. If you have to stay in bed, it is important to do regular leg movements and deep-breathing exercises. These help to reduce your risk of getting a blood clot or a chest infection. A physiotherapist will show you how to do these.

If you had a bigger operation, you may spend some time in intensive care or a high-dependency unit after the operation. You have one-to-one nursing care for as long as necessary to help you recover.

After surgery you may wake up with a number of drips, drains and tubes attached to you. These are gradually removed as you recover.

Drips

Operations to the mouth and throat area usually cause swelling. This can make eating and drinking difficult and uncomfortable for a time. You will usually have a tube going into a vein in your arm or neck (an intravenous drip). The nurses give you fluids through this tube for a few days. It is removed when you can drink fluids again.

Drains

Depending on the surgery, you may have 1 or 2 thin plastic drainage tubes coming from the operation area. The tubes have bottles attached to collect fluid from the wound. If you had reconstructive surgery, you may have a drain in the area where the tissue was taken from. Drains usually stay in place for 2 to 7 days. If you go home with a drain, the district nurse can check it. You usually come back to hospital to have it removed.

Your wound

The type of wound you have depends on the surgery. The nurses will check it regularly and change the dressings when needed. If you do not have dissolvable stitches, you usually have your stitches or staples removed about 7 days after surgery. The nurses will give you advice on what to do when you go home and how to recognise any signs of infection. District nurses can visit you at home and check your wound.

Feeding tube

A [feeding tube](#) can be used if you are not able to eat or drink in the usual way. If you are likely to need this, your team will discuss it with you. The tube may be put in before or during your operation.

You may have a thin tube passed through the nose and into the stomach. This is called a nasogastric tube. It can usually be removed when you are able to swallow.

Having a feeding tube into your stomach means you get the nutrition you need while your tissue heals. Your dietitian prescribes high-protein, high-calorie, liquid food, to give through the tube.

Once your mouth and throat have healed, your speech and language therapist (SLT) may do a [swallowing](#) assessment. When you can eat and swallow safely, they remove the feeding tube. If you are having [chemoradiation](#) or [radiotherapy](#) after surgery, you may need to keep the tube in until all your treatment is finished.

If you need to go home with a feeding tube, your head and neck team will teach you and your family or friends how to look after it. You can also have daily home visits from community nutritional care nurses. They can help with feeding tube care and setting up the feeds.

If you have a tracheostomy tube

The swelling from your operation usually goes down after about 5 to 7 days and you can then breathe easily. When this happens, the [tracheostomy](#) tube is taken out. The opening is left to heal over naturally. You usually cannot speak with the tube in place. Your nurse or SLT will have talked to you about how best to communicate with others. This could be using writing materials, picture boards, a mobile phone, laptop or tablet.

Possible side effects of surgery

Possible side effects of surgery for head and neck cancer may include:

Pain



You may have pain or discomfort for a few days or weeks after your operation. Your doctor or nurse will explain how to manage the pain. Always tell your doctor or nurse if you are still in pain. They can increase the dose or change the painkillers. We have more information about [managing pain](#).

Changes in sensation or numbness



Surgery may affect the feeling in your mouth, face, ears, neck or shoulders. Some areas may feel numb. This can happen if nerves are bruised during the operation. It may take several months for the nerves to heal and sensation to come back.

The skin around any scars can feel numb. It may take several months for normal sensation to come back.

If a cancer is growing very close to a nerve, the only way to remove all the cancer is by cutting the nerve. If this happens, the changes may be permanent. We have more information about [numbness and changes in sensation](#).

Speech



Sometimes surgery to the mouth and throat can affect the way you speak. The throat, nose, mouth, tongue, teeth, lips and soft palate are involved in producing speech. Any operation that changes one of these areas may affect your speech or voice, or both. For some people, this is hardly noticeable. But for others, their speech or voice is temporarily or permanently changed.

An SLT can help you adapt to any changes and improve your communication. We have more information about managing [changes to speech and voice](#).

Swallowing



When you are ready to start having fluids and food by mouth, you may see an SLT. They can give you advice about foods that are easy to chew and swallow. If you cough when you eat and drink, it may be a sign that food is going down the wrong way (into your airway). Your SLT can teach you exercises to prevent this and [help with swallowing](#).

Difficulty opening your mouth due to a stiff jaw (trismus)



Some operations to the back of the mouth and throat can cause a [stiff jaw](#). This can make it difficult to open your mouth (trismus). It is usually temporary. There are exercises you can do to help prevent this from becoming a permanent problem.

Changes to your appearance



Before surgery, your surgeons and nurse will talk to you about any possible changes to your appearance. It is important that you have a good idea of what to expect.

Swelling



Surgery to the mouth or throat can often cause swelling. Your face and neck may look very swollen immediately after the operation, but this slowly gets better over a few months.

Scars



Your surgeon tries as much as possible to make sure any scars are in less noticeable places. For example, they could be in skin creases on your face or a fold in your neck.

Scars are usually red or dark to begin with but fade over time. Your nurse will give you advice on what to expect after surgery. Using camouflage products can also help.

It takes time to adjust to changes in how you look. There is support to help you. We have more information about [coping with changes in your appearance](#).

Going home

How long you are in hospital for will depend on the type of surgery you had and your recovery. It may be 2 to 4 weeks if you had reconstructive surgery.

You will have an appointment to come back to the outpatient clinic a few weeks later to see your surgeon. You will also be given appointments if you need to see any other members of the team, such as your speech and language therapist (SLT), specialist nurse or dietitian.

At the outpatient appointment with your surgeon, they will check that your wound is healing and you are recovering well. They will talk to you about the results of your operation and any more treatment you might need.

You can also ask any questions you have.

Recovering from surgery for head and neck cancer

It may take weeks or months to recover, depending on the type of operation you have had. You may be coping with different side effects from your surgery.

Recovery takes time and it is faster for some people than others. Try to pace yourself and do not do too much too soon.

Your doctor and nurse will give you advice on what you can do to help your recovery. Try to eat healthily and get enough rest. You can slowly build up your strength and fitness with light exercise, such as short walks.

Driving

Your doctor will tell you when it is safe to drive after your operation. If you had surgery to your neck, it can take several weeks for you to be fit enough to drive.

Coping with other changes

We have more information about managing different changes after treatment. This includes possible effects on:

- [eating and drinking](#)
- your [speech and voice](#)
- your [appearance](#)
- your [sex life](#) and relationships.

About our information

References



Below is a sample of the sources used in our head and neck cancer information. If you would like more information about the sources we use, please contact us at cancerinformationteam@macmillan.org.uk

Machiels J.-P, Leemans C. R. et al. Squamous cell carcinoma of the oral cavity, larynx, oropharynx and hypopharynx. EHNS- ESMO-ESTRO Clinical Practice Guidelines for diagnosis, treatment and follow-up. Annals of Oncology, 2020. Volume 31, Issue 11, Pages 1462-1475.

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, Dr Chris Alcock, Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

Date reviewed

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ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help